DIAGNOSTIC REPORT

MC-5718

agilus diagnostics | Formerly SRL
Dr. Phadke Labs

PATIENT NAME : HEMA GANDHI **REF. DOCTOR :** SELF

CODE/NAME & ADDRESS : C000157515
AGILUS DIAGNOSTICS POONAM NAGAR PSC- HOME SHOP NO 13, C-8 POONAM DARSHAN BEHIND OFF MAHAKALI ROAD,POONAM NAGAR
MUMBAI 400093
022-46626009

ACCESSION NO : **0002YG026292**
PATIENT ID : HEMAF2103570
CLIENT PATIENT ID: HEMAF2103570
ABHA NO :

AGE/SEX : 68 Years Female
DRAWN : 15/07/2025 09:17:39
RECEIVED : 15/07/2025 12:24:59
REPORTED : 15/07/2025 16:48:20

Test Report Status **Final**	Results	Biological Reference Interval	Units

HAEMATOLOGY - CBC

CBC-5, EDTA WHOLE BLOOD

BLOOD COUNTS, EDTA WHOLE BLOOD

HEMOGLOBIN (HB) METHOD : CYANIDE FREE COLORIMETRIC DETERMINATION	12.6	12.0 - 15.0	g/dL
RED BLOOD CELL (RBC) COUNT METHOD : ELECTRICAL IMPEDANCE	4.25	3.8 - 4.8	mil/µL
WHITE BLOOD CELL (WBC) COUNT METHOD : FLUORESCENT FLOW CYTOMETRY	5.98	4.0 - 10.0	thou/µL
PLATELET COUNT METHOD : ELECTRICAL IMPEDENCE & MICROSCOPY	160	150 - 410	thou/µL

RBC AND PLATELET INDICES

HEMATOCRIT (PCV) METHOD : CALCULATED PARAMETER	38.9	36 - 46	%
MEAN CORPUSCULAR VOLUME (MCV) METHOD : DERIVED PARAMETER FROM RBC HISTOGRAM	91.6	83 - 101	fL
MEAN CORPUSCULAR HEMOGLOBIN (MCH) METHOD : CALCULATED PARAMETER	29.6	27.0 - 32.0	pg
MEAN CORPUSCULAR HEMOGLOBIN CONCENTRATION(MCHC) METHOD : CALCULATED PARAMETER	32.4	31.5 - 34.5	g/dL
RED CELL DISTRIBUTION WIDTH (RDW) METHOD : DERIVED PARAMETER FROM RBC HISTOGRAM	**14.7 High**	11.6 - 14.0	%
MEAN PLATELET VOLUME (MPV) METHOD : DERIVED PARAMETER FROM PLATELET HISTOGRAM	**12.2 High**	6.8 - 10.9	fL

WBC DIFFERENTIAL COUNT

NEUTROPHILS METHOD : FLUORESCENT FLOW CYTOMETRY	74	40 - 80	%
LYMPHOCYTES METHOD : FLUORESCENT FLOW CYTOMETRY	**14 Low**	20 - 40	%
MONOCYTES METHOD : FLUORESCENT FLOW CYTOMETRY	9	2 - 10	%

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Dr. Deepak Sanghavi
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EOSINOPHILS METHOD : FLUORESCENT FLOW CYTOMETRY	3	1 - 6	%
BASOPHILS METHOD : FLUORESCENT FLOW CYTOMETRY	0	0 - 2	%
ABSOLUTE NEUTROPHIL COUNT METHOD : FLUORESCENT FLOW CYTOMETRY / MICROSCOPY, CALCULATED	4.39	2.0 - 7.0	thou/µL
ABSOLUTE LYMPHOCYTE COUNT METHOD : FLUORESCENT FLOW CYTOMETRY / MICROSCOPY, CALCULATED	**0.86 Low**	1.0 - 3.0	thou/µL
ABSOLUTE MONOCYTE COUNT METHOD : FLUORESCENT FLOW CYTOMETRY / MICROSCOPY, CALCULATED	0.52	0.2 - 1.0	thou/µL
ABSOLUTE EOSINOPHIL COUNT METHOD : FLUORESCENT FLOW CYTOMETRY / MICROSCOPY, CALCULATED	0.16	0.02 - 0.50	thou/µL
ABSOLUTE BASOPHIL COUNT METHOD : FLUORESCENT FLOW CYTOMETRY / MICROSCOPY, CALCULATED	0	0.0 - 0.1	thou/µL

Interpretation(s)
RBC AND PLATELET INDICES-Mentzer index (MCV/RBC) is an automated cell-counter based calculated screen tool to differentiate cases of Iron deficiency anaemia(>13) from Beta thalassaemia trait (<13) in patients with microcytic anaemia. This needs to be interpreted in line with clinical correlation and suspicion. Estimation of HbA2 remains the gold standard for diagnosing a case of beta thalassaemia trait.
WBC DIFFERENTIAL COUNT-The optimal threshold of 3.3 for NLR showed a prognostic possibility of clinical symptoms to change from mild to severe in COVID positive patients. When age = 49.5 years old and NLR = 3.3, 46.1% COVID-19 patients with mild disease might become severe. By contrast, when age < 49.5 years old and NLR < 3.3, COVID-19 patients tend to show mild disease. (Reference to - The diagnostic and predictive role of NLR, d-NLR and PLR in COVID-19 patients A.-P. Yang, et al. International Immunopharmacology 84 (2020) 106504
This ratio element is a calculated parameter and out of NABL scope.

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COAGULATION			
PROTHROMBIN TIME, CITRATE PLASMA			
PROTHROMBIN TIME (PT) METHOD : CLOT-BASED ASSAY	15.8	12.2 - 16.0	SECONDS
INTERNATIONAL NORMALIZED RATIO (INR)	1.13	0.86 - 1.14	RATIO
MEAN PROTHROMBIN TIME OF CONTROL PLASMA (MNPT)	14.1		SECONDS

Interpretation(s)
PROTHROMBIN TIME, CITRATE PLASMA-**TEST DESCRIPTION**- Prothrombin Time measures the integrity of the extrinsic pathway and the adequacy of critical coagulation factors involved in it, namely, Factor VII. This test is therefore, used for monitoring oral anticoagulation therapy which lowers the levels of multiple vitamin K dependent coagulation factors in blood (Factors II, VII, IX and X) including Factor VII. The result of PT is expressed as International Normalized Ratio (INR) to neutralize the influence of variable sensitivity of the reagents (thromboplastin) used in the assay by different laboratories
TEST INTERPRETATION- INCREASED PT may be due to:
1.Factor deficiencies,2. Drugs (e.g. Coumarin-type drugs for anticoagulant therapy, salicytes),3. Severe liver damage (e.g. poisoning, hepatitis, cirrhosis),4.Hypofibrinogenemia (Acquired or Inherited),5.Hemorrhagic disease of the newborn.,6 Poor fat absorption (e.g.obstructive jaundice,fistulas,sprue,steatorrhea,celiac disease,colitis,chronic diarrhoea.)
RECOMMENDATION-This is a very sensitive reagent and therefore it is advisable of follow up I.N.R. value rather than P.T. in seconds.The recommended I.N.R. is 2 - 3 for patients on oral anticoagulant in all conditions except mechanical valve replacement and prevention of myocardial infarction where the I.N.R. may be maintained between 2.5-3.5. Please stop anticoagulant therapy if the I.N.R. is > 4.5.

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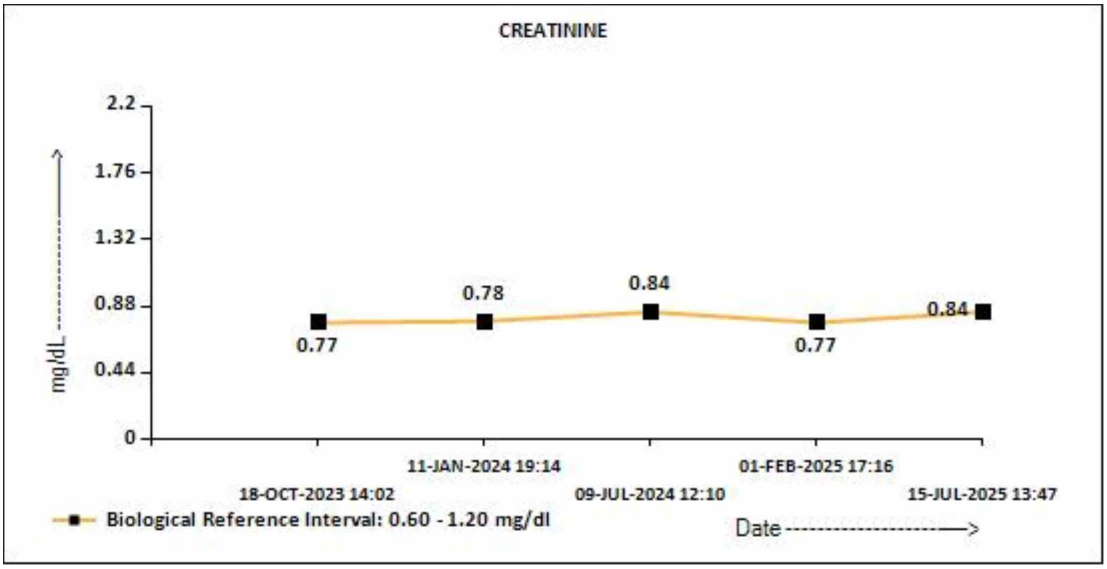
BIOCHEMISTRY

CREATININE, SERUM

CREATININE	0.84	0.60 - 1.20	mg/dL

METHOD : SPECTROPHOTOMETRY, JAFFE'S ALKALINE PICRATE KINETIC - RATE BLANKED - IFCC-IDMS STANDARIZED



ELECTROLYTES (NA/K/CL), SERUM

SODIUM, SERUM	**133 Low**	135 - 145	mmol/L
METHOD : ISE INDIRECT			
POTASSIUM, SERUM	**3.10 Low**	3.5 - 5.3	mmol/L
METHOD : ISE INDIRECT			
CHLORIDE, SERUM	**93 Low**	97 - 110	mmol/L
METHOD : ISE INDIRECT			

Interpretation(s)
CREATININE, SERUM-**Higher than normal level may be due to:**
• Blockage in the urinary tract, Kidney problems, such as kidney damage or failure, infection, or reduced blood flow, Loss of body fluid (dehydration), Muscle problems, such

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as breakdown of muscle fibers, Problems during pregnancy, such as seizures (eclampsia)), or high blood pressure caused by pregnancy (preeclampsia)
Lower than normal level may be due to:• Myasthenia Gravis, Muscuophy

****End Of Report****
Please visit www.agilusdiagnostics.com for related Test Information for this accession

CONDITIONS OF LABORATORY TESTING & REPORTING

1. It is presumed that the test sample belongs to the patient named or identified in the test requisition form.
2. All tests are performed and reported as per the turnaround time stated in the AGILUS Directory of Services.
3. Result delays could occur due to unforeseen circumstances such as non-availability of kits / equipment breakdown / natural calamities / technical downtime or any other unforeseen event.
4. A requested test might not be performed if:
   i. Specimen received is insufficient or inappropriate
   ii. Specimen quality is unsatisfactory
   iii. Incorrect specimen type
   iv. Discrepancy between identification on specimen container label and test requisition form
5. AGILUS Diagnostics confirms that all tests have been performed or assayed with highest quality standards, clinical safety & technical integrity.
6. Laboratory results should not be interpreted in isolation; it must be correlated with clinical information and be interpreted by registered medical practitioners only to determine final diagnosis.
7. Test results may vary based on time of collection, physiological condition of the patient, current medication or nutritional and dietary changes. Please consult your doctor or call us for any clarification.
8. Test results cannot be used for Medico legal purposes.
9. In case of queries please call customer care (91115 91115) within 48 hours of the report.

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